CLINICAL CONSULTATION: Acute severe hematochezia in an 84-year-old female on apixaban for non-valvular atrial fibrillation.

HPI: The patient was admitted through the emergency department after passing three consecutive large-volume bowel movements of maroon and bright red blood mixed with clots over the past 8 hours, accompanied by lightheadedness and diaphoresis. She denies hematemesis, prior melena, abdominal cramping, or history of nonsteroidal anti-inflammatory drug (NSAID) consumption. Her last dose of apixaban (5 mg) was taken 14 hours prior to admission.

EMERGENCY RESUSCITATION AND PHYSICAL EXAM:

Vitals on initial presentation: BP 88/54 mmHg, HR 108 bpm, SpO2 97% on room air. Following 2 liters of intravenous normal saline resuscitation and transfusion of two units of packed red blood cells, vital signs stabilized: BP 114/70 mmHg, HR 82 bpm.

Abdomen: Soft, non-distended, completely non-tender without guarding or rebound tenderness. Normal active bowel sounds.

Digital Rectal Examination: Voluminous maroon liquid stool with clotted blood visualized in the rectal vault; no palpable anorectal masses. Nasogastric tube lavage returned clear bilious fluid without blood, effectively ruling out rapid upper gastrointestinal hemorrhage.

Laboratories: Hemoglobin dropped from baseline 13.2 g/dL to 7.8 g/dL; Platelets 210,000/uL; INR 1.3; BUN elevated at 34 mg/dL with Creatinine 1.0 mg/dL (BUN/Cr ratio 34, typical for intravascular volume depletion and protein absorption).

URGENT COLONOSCOPY FINDINGS:

Following rapid polyethylene glycol purge, urgent colonoscopy was executed. The colonoscope was advanced to the cecum. Active arterial spurting hemorrhage was localized to a neck of a diverticulum located in the ascending colon at 75 cm from the anal verge. The mucosal field was cleared with endoscopic water jet irrigation. Through the working channel, two Boston Scientific Resolution endoclips were strategically deployed across the diverticular orifice and bleeding arterial vessel, resulting in immediate and complete hemostasis. The remainder of the colon revealed widespread sigmoid diverticulosis without active bleeding or stigmata of recent hemorrhage.

RECOMMENDATIONS:

1. Diverticular hemorrhage, right colon, successfully arrested with endoscopic clip application.

2. Hold apixaban for 48 hours; cardiology consult regarding risk-benefit stratification for restarting anticoagulation.

3. Maintain intravenous iron therapy; monitor hemoglobin every 8 hours.